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Applicable taxes levied as per extant tax laws shall be deducted from the premium or from the allotted units as applicable.

Disclaimer: IndiaFirst Life Insurance Company Limited, IRDAI Regn No.143, CIN: U66010MH2008PLC183679, Address: 12th & 13th floor, North [C] Wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400 063. www.indiafirstlife.com, SMS <LIFE> to 5667735 SMS Charges apply. Toll free No – 1800 209 8700. Trade logo of promoters displayed above belongs to M/s Bank of Baroda and is used by IndiaFirst Life Insurance Co. Ltd. under license.

BEWARE OF SPURIOUS PHONE CALLS AND FICTIOUS/ FRAUDULENT OFFERS

- IRDAI or its official do not involve in activities like selling insurance policies, announcing bonus or investment of premiums. Public receiving such phone calls are requested to lodge a policy complaint.

PART A

INDIAFIRST LIFE INSURANCE COMPANY LIMITED

Regd. & Corporate Office: 12th & 13th floor, North [C] Wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400 063. IRDAI Regn No.143.
CIN: U66010MH2008PLC183679

To,
XXXX XXXX
Address 1,
Address 2.
Pin code – xxx xxx

DD/MM/YYYY

IndiaFirst Life Group Critical Illness Rider – UIN: 143B002V02
(Non Linked, Non Participating, Group, Health, Pure Risk Rider)

Dear Customer,

Congratulations!

You have taken a step towards insuring Your Customer/Employee and We are glad to be a part of this journey with You.

We have provided you with the relevant information about your Rider policy in this policy document. Please read it carefully to ensure that this is the right policy for your financial needs.

You/Member have a free look period of 30 days beginning from the date of receipt of the Rider Policy/Certificate of Insurance(COI), whether received electronically or otherwise, to review the terms and conditions of the Rider Policy and in case You/Member disagree to any of those terms or conditions, You shall have the option to return the Rider Policy/COI to us for cancellation, stating the reasons for Your objection, provided that no claim has been made under the Rider Policy/COI. In such an event, You/Member shall be entitled to a refund of the Rider Premium paid subject only to a deduction of a proportionate risk premium for the period of cover and the expenses, if any, incurred by us on medical examination and stamp duty charges.

Such a request received by us for Free-Look cancellation of the Rider Policy shall be processed and Rider Premium shall be refunded within 7 days of receipt of the request, subject to applicable deductions.

Kindly refer to Customer Information Sheet (CIS) enclosed with this Policy Document for key information regarding your Policy.

In case of Employer-Employee Groups, only the Master Policyholder, and not the Member, shall have the right to exercise the Free-Look Cancellation option.

In case of any communication in respect of the policy; You may contact Us at IndiaFirst Life Insurance Company Ltd, 12th & 13th floor, North [C] Wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400 063. You can also write to Us at corporate.service@indiafirstlife.com or contact us on 1800 209 8700.

Thank you once again for choosing IndiaFirst Life.

Yours truly,

Authorised Signatory

Insurance Agent/ Intermediary Details

Name:	
Agent/Intermediary Code:	
Telephone No.:	
Address:	
E-mail ID :	

IndiaFirst Life Group Critical Illness Rider UIN: 143B002V02
(Non-Linked Non-Participating Group, Health, Pure Risk Rider)

XXX_____ (hereinafter called the "Master Policyholder") have by a written Proposal Form dated _____ requested the IndiaFirst Life Insurance Company Limited (hereinafter called the "Insurer") to grant the benefits of insurance cover under IndiaFirst Life Group Critical Illness Rider and as per the Scheme Rules of the (.....name of the scheme....) Scheme of the Master Policyholder (certified copy of which has been furnished to the Insurer by the Master Policyholder) to the Members whose names have been recorded in the Membership Register maintained by the Master Policyholder.

The Master Policyholder has also furnished to the Insurer statements containing the age and other details of each Member for whose benefit the rider Policy hereunder is being effected. The Master Policyholder and the Insurer have accepted and agreed that the said Proposal Form, Member Application Form, certified copy of the Scheme Rules along with other statements (as applicable) and other supporting documents leading to the issuance of this Rider Policy/COI shall be the basis of the contract of insurance.

If any of the details of the Member contained in the statement submitted by the Master Policyholder/Member are incomplete, false or incorrect, coverage in respect of such Member under the Rider Policy/COI shall be cancelled by Us in accordance with Section 45 of the Insurance Act, 1938 as amended from time to time.

It is further hereby declared that every endorsement placed on the Rider Policy by the Company shall be deemed part of the Policy Document.

Signed by and on behalf of

IndiaFirst Life Insurance Company Limited

Authorised Signatory

Annexure A - Rider Policy Schedule

Rider Policy Details

IndiaFirst Life Group Critical Illness Rider is a Non-linked, Non-participating, Group, Health, Pure Risk Rider.

I. Master Policyholder Details

Master Policyholder Name:	
Address:	
Master Policy Number:	
Rider Policy Number:	
Name of the Scheme:	

II. Base Policy Details <<as per the Base Policy>>

III. Rider Details

Rider Name:	IndiaFirst Life Group Critical Illness Rider
UIN:	143B002V02
Benefit Option Opted:	<< 1. Critical Illness (CI) Benefit with 40 conditions 2. Critical Illness (CI) Benefit with 20 conditions 3. Critical Illness (CI) Benefit with 5 conditions>>
Eligibility condition	Minimum age at Entry: 18 years Maximum age at Entry: 69 years for One-year renewable Group Term insurance plan; 70 years for Group Credit life insurance plan

Rider Name (Benefit Option)	Total number of members covered	Rider Policy Commencement Date	Free Cover Limit	Total Rider Sum Insured (in INR)	Rider Cover Term	Rider Premium Paying Term	Rider Premium Payment Mode	Rider Premium (in INR)	Applicable Taxes (in INR)	Total Rider Premium (including Applicable Taxes) in INR

IV. Insurance Agent/Intermediary Details

Name:	
Agent/Intermediary Code:	
Telephone No.:	
Address:	
E-mail ID :	

V. Special Conditions

NIL	

The stamp duty of INR_____ (Rupees in words only) paid by pay order, vide receipt no._____ dated _____.Government Notification Revenue and Forest Department No. Mudrank 2004/415/CR/690/M-1, dated 31.12.2004

Note: ON EXAMINATION OF THIS POLICY, if you notice any mistake, then, you may contact us for correction of the same.

The Premium payable under this Policy may differ on the basis of the Extra Premiums, if any, the Premium payment mode chosen by you and the applicable Modal Factor.

Please read the terms and conditions of this Policy carefully to understand the terms referred to in this Policy Schedule.

PART B

Definitions

We have listed below a few words, terms and phrases which have been used in this Policy along with their meaning for your easy reference.

Word	Meaning
Age	Age of the member as at the last birthday on the Entry Date and on any subsequent Rider Policy annual renewal date as the case may be .
Annexure	Any annexure, endorsement attached to this Policy as changed/ modified and issued by us from time to time.
Annualized Premium	An amount which is payable in a Policy Year, excluding Extra Premium, loadings for modal premiums and applicable taxes, cesses or levies, if any.
Appointee	The person appointed by the Member to receive the benefits under this Rider Policy, if the Nominee is a Minor.
Beneficiary/Claimant	Refers to the person who is entitled to receive benefits under this Rider Policy. The beneficiary may be Member or his Assignee or Nominee or other Legal Representatives as the case may be.
Base Policy	The group life insurance policy to which this Rider Policy is attached.
Certificate of Insurance (COI)	A document stating the benefits payable to the Member and other details pertaining to the Coverage of the Member under this Rider Policy.
Customer Information Sheet (CIS)	To ensure that Master Policyholder/Members are well informed about their Coverage, rights, and responsibilities, the Company provides a detailed document, known as the 'Customer Information Sheet' along with the COI.
Entry Date	The date on which the Member joins the Scheme under the Scheme Rules. This may or may not be the same as the Rider Policy Commencement Date.
Extra Premium	An additional amount payable by you, which is determined by us in accordance with our board approved underwriting policy. This is determined on the basis of information provided by you in the Proposal Form or on the basis of any other information submitted to us or through medical examination of the member subject to your consent.
Free Look Period	A period of 30 days from the date of receipt of the Rider Policy/COI,
Grace Period	Grace period means the specified period of time immediately following the premium due date during which a payment can be made to renew or continue a the Rider policy in force without loss of continuity benefits such as waiting periods and coverage of pre-existing diseases. If a valid claim has occurred during grace period, then rider Sum Assured will be paid after deducting due Rider premium. Grace period terms and conditions will be applicable under Regular/ Limited premium payment options.
Income Tax Act	Income Tax Act, 1961, as amended from time to time
Installment Premium	An amount that you pay us during the Premium Paying Term at regular intervals for securing the benefits under this Policy.
Insurance Act	Insurance Act, 1938 and as amended from time to time
Lapse	Non-payment of Rider Premium within the expiry of grace period.
Master Policyholder	Master Policyholder is the trustee or organization that effects this Rider Policy for the benefit of its Members. The Master Policyholder holds this Rider Policy.
Member	A person who meets the eligibility criteria specified in the Scheme Rules and whose name has been recorded under the Scheme and this Rider Policy as a Member effective from the Entry Date after due approval from the Company and with respect to whom the health insurance cover under this Rider Policy has been effected.
Member Application Form	The application form to be submitted by an eligible person to become a Member under this Rider Policy.

Word	Meaning
Member Risk Commencement Date	The date from which the Rider Cover for a Member is activated under this Rider Policy. Under Non-Employer-Employee cases, this date is mentioned on the Certificate of Insurance (COI). Under Employer-Employee cases, this date is determined as per Scheme rules.
Nominee	Nominee is the person nominated by the member under the Base Policy, as per Section 39 of the Insurance Act, who is authorized to receive the claim benefit payable under this Rider Policy and to give a valid discharge to the Company on settlement of the claim
Rider Policy or Policy Document	The IndiaFirst Life Group Critical Illness Rider which includes this Rider Policy wording (as may be changed/ modified by us subject to receipt of prior approval of the Regulatory Authority, from time to time), the Proposal Form, Annexures, the Rider Policy Schedule, any tables, information and documents which form a part of this Rider Policy.
Rider Cover	The coverage of risk of the Member on happening of any of the events covered under the benefit option chosen by the Master Policyholder/ Member, as applicable, under this Rider Policy.
Rider Policy Commencement Date	The date on which this Rider Policy is issued by us. This is specified in the Rider Policy Schedule.
Rider Policy Schedule	The schedule contained in Part A of this Policy Document and if we have issued a revised Policy Schedule, then, such revised Rider Policy Schedule.
Rider Policy Term	Rider Policy Term means the period mentioned in the Rider Policy Schedule commencing from the Rider Policy Commencement Date
Rider Premium	The amount payable by the Master Policyholder/Member for securing the benefits under this Rider Policy. It does not include Extra Premium or applicable taxes.
Rider Premium Paying Term	The time period during which you need to pay your Premiums regularly to us for securing the benefits under this Policy.
Proposal Form	The proposal form completed and signed by the Mater Policyholder based on which we have issued this Rider Policy .
Rider Cover Term	A period starting from Rider Policy Commencement Date or Entry Date, whichever is later, and ending on the expiry of this Rider Policy Term.
Risk Commencement Date	The date on which the insurance coverage starts under this Policy. This is specified in the Policy Schedule.
Regulatory Authority	The Insurance Regulatory and Development Authority of India (IRDAI) or such other authority or authorities, as may be designated/ appointed under the applicable laws and regulations as having the authority to oversee and regulate life insurance business in India.
Revival	Revival is the process of restoring the benefits under the Rider Policy which are otherwise not available due to the nonpayment of Rider premiums on due dates, resulting in the Rider Policy getting lapsed.
Revival Period	The Revival period shall be same as the base policy to which this Rider Policy is attached during which you can pay the due unpaid Rider Premiums without any interest to us and comply with the conditions specified in Part D, as the case may be for reviving the Rider Policy.
Rider Sum Assured	The Rider Sum Assured refers to the amount of Rider Cover provided under this Rider Policy, which is distinct from the benefits provided under the Base Policy, as per the benefit option chosen by the Master Policyholder/ Member at the inception of Rider Cover.
Scheme	Means a Scheme framed / formulated by the Master Policyholder to provide insurance cover to its Members.
Scheme Rules	Rules applicable to a Scheme of the Master Policyholder.
Surrender	Surrender means complete withdrawal or termination of this Rider Policy.
Surrender Value	Surrender Value means an amount, if any, that becomes payable on Surrender of a Policy during the Rider Policy Term, in accordance with the terms and conditions of this Rider Policy.

Word	Meaning
We or us or our or Insurer or Company	IndiaFirst Life Insurance Company Limited.
You or your or Policyholder or Proposer	The Master Policyholder
Definitions	
Medical Practitioner	Medical Practitioner means a person who holds a valid registration from the Medical Council of any State or Medical Council of India or Council for Indian Medicine or for Homeopathy set up by the Government of India or a State Government and is thereby entitled to practice medicine within its jurisdiction; and is acting within its scope and jurisdiction of license. The Medical practitioner should not be the The Master policyholder/ Member himself/herself; or an authorized insurance intermediary (or related persons) involved with selling or servicing the insurance contract in question; or employed by or under contractual engagement with the insurance company; related to the Master policyholder/ Member by blood or marriage
Pre-Existing disease	Pre-Existing disease means any condition, ailment, injury or disease: a. That is/are diagnosed by a Medical Practitioner not more than 36 months prior to the Member Risk Commencement Date or its reinstatement; or b. For which medical advice or treatment was recommended by, or received from, a Medical Practitioner not more than 36 months prior to the Member Risk Commencement Date or its reinstatement; or c. A condition for which any symptoms and or signs if presented and have resulted within three months of the issuance of the Rider Policy in a diagnostic illness or medical condition. After completion of 36 months from the Member Risk Commencement Date or reinstatement, as the case may be, Pre-Existing Disease exclusion clause will not be applicable.
Definitions with disease specific exclusions of Critical Illness are given below. Details of the benefits are provided in Part C	
Waiting Period	There will be a waiting period of 90 days from Member Risk Commencement Date, or from any subsequent Revival/reinstatement, whichever is later. No benefits are payable with respect to any Critical Illness diagnosed within the Waiting Period.
Survival Period	There will be a survival period of 28 days applicable from the date of diagnosis of a critical illness. The Member must survive during this period to be eligible for critical illness benefit payment.

Word	Meaning
Cancer of Specified Severity	I. A malignant tumor characterized by the uncontrolled growth and spread of malignant cells with invasion and destruction of normal tissues. This diagnosis must be supported by histological evidence of malignancy. The term cancer includes leukemia, lymphoma and sarcoma. II. The following are excluded – i. All tumors which are histologically described as carcinoma in situ, benign, pre-malignant, borderline malignant, low malignant potential, neoplasm of unknown behavior, or non-invasive, including but not limited to: Carcinoma in situ of breasts, Cervical dysplasia CIN-1, CIN -2 and CIN-3. ii. Any non-melanoma skin carcinoma unless there is evidence of metastases to lymph nodes or beyond; iii. Malignant melanoma that has not caused invasion beyond the epidermis; iv. All tumors of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least clinical TNM classification T2N0M0 v. All Thyroid cancers histologically classified as T1N0M0 (TNM Classification) or below; vi. Chronic lymphocytic leukaemia less than RAI stage 3 vii. Non-invasive papillary cancer of the bladder histologically described as TaN0M0 or of a lesser classification, viii.All Gastro-Intestinal Stromal Tumors histologically classified as T1N0M0 (TNM Classification) or below and with mitotic count of less than or equal to 5/50 HPFs;
Open Chest CABG	I. The actual undergoing of heart surgery to correct blockage or narrowing in one or more coronary artery(s), by coronary artery bypass grafting done via a sternotomy (cutting through the breast bone) or minimally invasive keyhole coronary artery bypass procedures. The diagnosis must be supported by a coronary angiography and the realization of surgery has to be confirmed by a cardiologist. II. The following are excluded: i. Angioplasty and/or any other intra-arterial procedures
Myocardial Infarction (First Heart Attack of specific severity)	I. The first occurrence of heart attack or myocardial infarction, which means the death of a portion of the heart muscle as a result of inadequate blood supply to the relevant area. The diagnosis for Myocardial Infarction should be evidenced by all of the following criteria: i. A history of typical clinical symptoms consistent with the diagnosis of acute myocardial infarction (For e.g. typical chest pain) ii. New characteristic electrocardiogram changes iii. Elevation of infarction specific enzymes, Troponins or other specific biochemical markers. II. The following are excluded: i. Other acute Coronary Syndromes ii. Any type of angina pectoris iii. A rise in cardiac biomarkers or Troponin T or I in absence of overt ischemic heart disease OR following an intra-arterial cardiac procedure
Open Heart Replacement or repair of heart valves	I. The actual undergoing of open-heart valve surgery is to replace or repair one or more heart valves, as a consequence of defects in, abnormalities of, or disease affected cardiac valve(s). The diagnosis of the valve abnormality must be supported by an echocardiography and the realization of surgery has to be confirmed by a specialist medical practitioner. Catheter based techniques including but not limited to, balloon valvotomy / valvuloplasty are excluded.

Word	Meaning
Primary (Idiopathic)Pulmonary Hypertension	I. An unequivocal diagnosis of Primary (Idiopathic) Pulmonary Hypertension by a Cardiologist or specialist in respiratory medicine with evidence of right ventricular enlargement and the pulmonary artery pressure above 30 mm of Hg on Cardiac Cauterization. There must be permanent irreversible physical impairment to the degree of at least Class IV of the New York Heart Association Classification of cardiac impairment. II. The NYHA Classification of Cardiac Impairment are as follows: i. Class III: Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes symptoms. ii. Class IV: Unable to engage in any physical activity without discomfort. Symptoms may be present even at rest. III. Pulmonary hypertension associated with lung disease, chronic hypoventilation, pulmonary thromboembolic disease, drugs and toxins, diseases of the left side of the heart, congenital heart disease and any secondary cause are specifically excluded
Kidney Failure requiring regular dialysis	I. End stage renal disease presenting as chronic irreversible failure of both kidneys to function, as a result of which either regular renal dialysis (haemodialysis or peritoneal dialysis) is instituted or renal transplantation is carried out. Diagnosis has to be confirmed by a specialist medical practitioner.
Major Organ/ Bone Marrow Transplant	I. The actual undergoing of a transplant of: i. One of the following human organs: heart, lung, liver, kidney, pancreas, that resulted from irreversible end-stage failure of the relevant organ, or ii. Human bone marrow using haematopoietic stem cells. The undergoing of a transplant has to be confirmed by a specialist medical practitioner. II. The following are excluded: i. Other stem-cell transplants ii. Where only islets of langerhans are transplanted
Stroke resulting in Permanent symptoms	I. Any cerebrovascular incident producing permanent neurological sequelae. This includes infarction of brain tissue, thrombosis in an intracranial vessel, haemorrhage and embolisation from an extracranial source. Diagnosis has to be confirmed by a specialist medical practitioner and evidenced by typical clinical symptoms as well as typical findings in CT Scan or MRI of the brain. Evidence of permanent neurological deficit lasting for at least 3 months has to be produced. II. The following are excluded: i. Transient ischemic attacks (TIA) ii. Traumatic injury of the brain iii. Vascular disease affecting only the eye or optic nerve or vestibular functions.
Benign Brain Tumour	I. Benign brain tumor is defined as a life threatening, non-cancerous tumor in the brain, cranial nerves or meninges within the skull. The presence of the underlying tumor must be confirmed by imaging studies such as CT scan or MRI. II. This brain tumor must result in at least one of the following and must be confirmed by the relevant medical specialist. i. Permanent Neurological deficit with persisting clinical symptoms for a continuous period of at least 90 consecutive days or ii. Undergone surgical resection or radiation therapy to treat the brain tumor. III. The following conditions are excluded: Cysts, Granulomas, malformations in the arteries or veins of the brain, hematomas, abscesses, pituitary tumors, tumors of skull bones and tumors of the spinal cord.
Coma of specified severity	I. A state of unconsciousness with no reaction or response to external stimuli or internal needs. This diagnosis must be supported by evidence of all of the following: i. no response to external stimuli continuously for at least 96 hours; ii. life support measures are necessary to sustain life; and iii. permanent neurological deficit which must be assessed at least 30 days after the onset of the coma. II. The condition has to be confirmed by a specialist medical practitioner. Coma resulting directly from alcohol or drug abuse is excluded.

Word	Meaning
End Stage Liver Disease	I. Permanent and irreversible failure of liver function that has resulted in all three of the following: i. Permanent jaundice; and ii. Ascites; and iii. Hepatic encephalopathy. II. Liver failure secondary to drug or alcohol abuse is excluded.
End Stage Lung Disease	I. End stage lung disease, causing chronic respiratory failure, as confirmed and evidenced by all of the following: i. FEV1 test results consistently less than 1 litre measured on 3 occasions 3 months apart; and ii. Requiring continuous permanent supplementary oxygen therapy for hypoxemia; and iii. Arterial blood gas analysis with partial oxygen pressure of 55mmHg or less (PaO2 < 55mmHg); and iv. Dyspnea at rest.
Loss of Limbs	The physical separation of two or more limbs, at or above the wrist or ankle level limbs as a result of injury or disease. This will include medically necessary amputation necessitated by injury or disease. The separation has to be permanent without any chance of surgical correction. Loss of Limbs resulting directly or indirectly from self-inflicted injury, alcohol or drug abuse is excluded.
Blindness	I. Total, permanent and irreversible loss of all vision in both eyes as a result of illness or accident. II. The Blindness is evidenced by: i. corrected visual acuity being 3/60 or less in both eyes or ; ii. the field of vision being less than 10 degrees in both eyes. III. The diagnosis of blindness must be confirmed and must not be correctable by aids or surgical procedure
Third Degree Burns	I. There must be third-degree burns with scarring that cover at least 20% of the body's surface area. The diagnosis must confirm the total area involved using standardized, clinically accepted, body surface area charts covering 20% of the body surface area
Major Head Trauma	I. Accidental head injury resulting in permanent Neurological deficit to be assessed no sooner than 3 months from the date of the accident. This diagnosis must be supported by unequivocal findings on Magnetic Resonance Imaging, Computerized Tomography, or other reliable imaging techniques. The accident must be caused solely and directly by accidental, violent, external and visible means and independently of all other causes. II. The Accidental Head injury must result in an inability to perform at least three (3) of the following Activities of Daily Living either with or without the use of mechanical equipment, special devices or other aids and adaptations in use for disabled persons. For the purpose of this benefit, the word "permanent" shall mean beyond the scope of recovery with current medical knowledge and technology. III. The Activities of Daily Living are: i. Washing: the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means; ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances; iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa; iv. Mobility: the ability to move indoors from room to room on level surfaces; v. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene; vi. Feeding: the ability to feed oneself once food has been prepared and made available. IV. The following are excluded: i. Spinal cord injury;

Word	Meaning
Permanent Paralysis of limbs	Total and irreversible loss of use of two or more limbs as a result of injury or disease of the brain or spinal cord. A specialist medical practitioner must be of the opinion that the paralysis will be permanent with no hope of recovery and must be present for more than 3 months.
Motor Neurone Disease with Permanent Symptoms	Motor neuron disease diagnosed by a specialist medical practitioner as spinal muscular atrophy, progressive bulbar palsy, amyotrophic lateral sclerosis or primary lateral sclerosis. There must be progressive degeneration of corticospinal tracts and anterior horn cells or bulbar efferent neurons. There must be current significant and permanent functional neurological impairment with objective evidence of motor dysfunction that has persisted for a continuous period of at least 3 months.
Multiple Sclerosis with Persistent Symptoms	I. The unequivocal diagnosis of Definite Multiple Sclerosis confirmed and evidenced by all of the following: i. investigations including typical MRI findings which unequivocally confirm the diagnosis to be multiple sclerosis and ii. there must be current clinical impairment of motor or sensory function, which must have persisted for a continuous period of at least 6 months. II. Neurological damage due to SLE is excluded
Deafness	I. Total and irreversible loss of hearing in both ears as a result of illness or accident. This diagnosis must be supported by pure tone audiogram test and certified by an Ear, Nose and Throat (ENT) specialist. Total means "the loss of hearing to the extent that the loss is greater than 90decibels across all frequencies of hearing" in both ears.
Loss of Speech	I. Total and irrecoverable loss of the ability to speak as a result of injury or disease to the vocal cords. The inability to speak must be established for a continuous period of 12 months. This diagnosis must be supported by medical evidence furnished by an Ear, Nose, Throat (ENT) specialist.
Surgery to Aorta	Undergoing of a laporotomy or thoracotomy to repair or correct an aneurysm, narrowing, obstruction or dissection of the aortic artery. For this definition, aorta means the thoracic and abdominal aorta but not its branches. Surgery performed using only minimally invasive or intra-arterial techniques such as percutaneous endovascular aneurysm repair are excluded
Apallic Syndrome	Universal necrosis of the brain cortex with the brain stem remaining intact. The definite diagnosis must be confirmed by a consultant neurologist and this condition has to be medically documented for at least one (1) month with no hope of recovery
Loss of Independent Existence	Loss of the physical ability through an illness or injury to do at least 3 of the 6 tasks listed below ever again. The relevant specialists must reasonably expect that the disability will last throughout life with no prospect of improvement, irrespective of when the cover ends or the Member expects to retire. The company's appointed doctor should also agree that the disability will last throughout life with no prospect of improvement, irrespective of when the cover ends or the Member expects to retire. The Member must need the help or supervision of another person and be unable to perform the task on their own, even with the use of special equipment routinely available to help and having taken any appropriate prescribed medication. The tasks are i. Washing: the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means; ii. Dressing: the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances; iii. Transferring: the ability to move from a bed to an upright chair or wheelchair and vice versa; iv. Mobility: the ability to move indoors from room to room on level surfaces; v. Toileting: the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene; vi. Feeding: the ability to feed oneself once food has been prepared and made available Loss of independent living must be medically documented for an uninterrupted period of at least six months. Proof of the same must be submitted to the Company while the Member is alive and permanently disabled. The company will have the right to evaluate the Member to confirm total and permanent disability. Loss of Independent Existence due to an injury should occur independently of any other causes within ninety (90) days of such injury. Coverage for this impairment will cease at age sixty-six (66) or on maturity data/expiry date, whichever is earlier.

Word	Meaning
Cardiomyopathy	The unequivocal diagnosis by a Consultant Cardiologist of Cardiomyopathy causing permanent impaired left ventricular function with an ejection fraction of less than 25%. This must result in severe physical limitation of activity to the degree of class IV of the New York Heart Classification and this limitation must be sustained over at least six months when stabilized on appropriate therapy. Cardiomyopathy directly related to alcohol or drug misuse is excluded. New York Heart Classification Class I. Patients with cardiac disease but without resulting limitation of physical activity. Ordinary physical activity does not cause undue fatigue, palpitation, dyspnea, or anginal pain. Class II. Patients with cardiac disease resulting in slight limitation of physical activity. They are comfortable at rest. Ordinary physical activity results in fatigue, palpitation, dyspnea, or anginal pain. Class III. Patients with cardiac disease resulting in marked limitation of physical activity. They are comfortable at rest. Less than ordinary activity causes fatigue, palpitation, dyspnea, or anginal pain. Class IV. Patients with cardiac disease resulting in inability to carry on any physical activity without discomfort. Symptoms of heart failure or the anginal syndrome may be present even at rest. If any physical activity is undertaken, discomfort increases.
Brain Surgery	The actual undergoing of surgery to the brain under general anaesthesia during which a craniotomy is performed. Keyhole surgery is included however, minimally invasive treatment where no surgical incision is performed to expose the target, such as irradiation by gamma knife or endovascular neuroradiological interventions such as embolizations, thrombolysis and stereotactic biopsy are excluded. Brain surgery as a result of an accident is also excluded. The procedure must be considered necessary by a qualified specialist."
Alzheimer`s Disease	Progressive and permanent deterioration of memory and intellectual capacity as evidenced by accepted standardised questionnaires and cerebral imaging. The diagnosis of Alzheimer's disease must be confirmed by an appropriate consultant and supported by the Company's appointed doctor. There must be significant reduction in mental and social functioning requiring the continuous supervision of the life assured. There must also be an inability of the Member to perform (whether aided or unaided) at least 3 of the following 5 "Activities of Daily Living" for a continuous period of at least 6 months: Activities of Daily Living are defined as: 1. Washing - the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means; 2. Dressing - the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances; 3. Transferring - the ability to move from a bed to an upright chair or wheelchair and vice versa; 4. Toileting - the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene; 5. Feeding - the ability to feed oneself once food has been prepared and made available. Psychiatric illnesses and alcohol related brain damage are excluded. Coverage for this impairment will cease at age sixty-six (66) or on maturity data/expiry date, whichever is earlier.
Muscular dystrophy	Muscular Dystrophy is a disease of the muscle causing progressive and permanent weakening of certain muscle groups. The diagnosis of muscular dystrophy must be made by a consultant neurologist, and confirmed with the appropriate laboratory, biochemical, histological, and electromyographic evidence. The disease must result in the permanent inability of the Member to perform (whether aided or unaided) at least three (3) of the five (5) "Activities of Daily Living". Activities of Daily Living are defined as: 1. Washing - the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means; 2. Dressing - the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances; 3. Transferring - the ability to move from a bed to an upright chair or wheelchair and vice versa; 4. Toileting - the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene; 5. Feeding - the ability to feed oneself once food has been prepared and made available

Word	Meaning
Parkinson`s Disease	The unequivocal diagnosis of idiopathic Parkinson's Disease by a consultant neurologist. This diagnosis must be supported by all of the following conditions: 1) The disease cannot be controlled with medication; and 2) There are objective signs of progressive deterioration; and 3) There is an inability of the Member to perform (whether aided or unaided) at least 3 of the following five (5) "Activities of Daily Living" for a continuous period of at least 6 months: Activities of Daily Living are defined as: 1. Washing - the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means; 2. Dressing - the ability to put on, take off, secure and unfasten all garments and, as appropriate, any braces, artificial limbs or other surgical appliances; 3. Transferring - the ability to move from a bed to an upright chair or wheelchair and vice versa; 4. Toileting - the ability to use the lavatory or otherwise manage bowel and bladder functions so as to maintain a satisfactory level of personal hygiene; 5. Feeding - the ability to feed oneself once food has been prepared and made available. Drug-induced or toxic causes of Parkinsonism are excluded. Coverage for this impairment will cease at age sixty-six (66) or on maturity data/expiry date, whichever is earlier
Medullary Cystic Disease	Medullary Cystic Disease is a disease where the following criteria are met: 1. The presence in the kidney of multiple cysts in the renal medulla accompanied by the presence of tubular atrophy and interstitial fibrosis; 2. Clinical manifestations of anaemia, polyuria and progressive deterioration in kidney function; and 3. The diagnosis of medullary cystic disease is confirmed by renal biopsy. Isolated or benign kidney cysts are specifically excluded from this benefit
Systemic Lupus Erythematosus	The unequivocal diagnosis by a consultant physician of systemic lupus erythematosus (SLE) with evidence of malar rash, discoid rash, photosensitivity, multi-articular arthritis, and serositis. There must also be hematological and immunological abnormalities consistent with the diagnosis of SLE. There must also be a positive antinuclear antibody test. There must also be evidence of central nervous system or renal impairment with either a) Renal involvement is defined as either persistent proteinuria greater than 0.5 grams per day or a spot urine showing 3+ or greater proteinuria b) Central nervous system involvement with permanent neurological dysfunction as evidenced with objective motor or sensory neurological abnormal signs on physical examination by a neurologist and present for at least 3 months. Seizures, headaches, cognitive and psychiatric abnormalities are not considered under this definition as evidence of "permanent neurological dysfunction". Discoid lupus and medication induced lupus are excluded
Aplastic Anaemia	Aplastic Anemia is chronic persistent bone marrow failure. A certified hematologist must make the diagnosis of severe irreversible aplastic anemia. There must be permanent bone marrow failure resulting in bone marrow cellularity of less than 25% and there must be two of the following: 1. Absolute neutrophil count of less than 500/mm^3 2. Platelets count less than 20,000/mm^3 3. Reticulocyte count of less than 20,000/mm^3 The Member must be receiving treatment for more than 3 consecutive months with frequent blood product transfusions, bone marrow stimulating agents, or immunosuppressive agents or the Member has received a bone marrow or cord blood stem cell transplant. Temporary or reversible aplastic anemia is excluded and not covered in this policy.
Poliomyelitis	The occurrence of Poliomyelitis where the following conditions are met: Poliovirus is identified as the cause; and Paralysis of the limb muscles or respiratory muscles must be present and persist for at least 3 months as confirmed by a consultant neurologist. Other causes of paralysis such as Guillain-Barre syndrome are specifically excluded

Word	Meaning
Bacterial Meningitis	Bacterial meningitis is a bacterial infection of the meninges of the brain causing brain dysfunction. There must be an unequivocal diagnosis by a consultant physician of bacterial meningitis that must be proven on analysis of the cerebrospinal fluid. There must also be permanent objective neurological deficit that is present on physical examination at least 3 months after the diagnosis of the meningitis infection
Encephalitis	Severe inflammation of the brain substance (cerebral hemisphere, brainstem or cerebellum) caused by viral infection and resulting in permanent neurological deficit. This diagnosis must be certified by a consultant neurologist and the permanent neurological deficit must be documented for at least 6 weeks.
Progressive Supra nuclear Palsy	Progressive supranuclear palsy occurring independently of all other causes and resulting in permanent neurological deficit, which is directly responsible for a permanent inability to perform at least two (2) of the Activities of Daily Living. The diagnosis of the Progressive Supranuclear Palsy must be confirmed by a registered Medical Practitioner who is a neurologist
Severe Rheumatoid Arthritis	The unequivocal diagnosis of Rheumatoid Arthritis must be made by a certified medical consultant based on clinically accepted criteria. There must be imaging evidence of erosions with widespread joint destruction in three or more of the following joint areas: hands, wrists, elbows, knees, hips, ankle, cervical spine or feet. There must also be typical rheumatoid joint deformities. Degenerative osteoarthritis and all other forms of arthritis are excluded. There must be history of treatment or current treatment with disease-modifying anti-rheumatic drugs, or DMARDs. Non-steroidal anti-inflammatory drugs such as acetylsalicylic acid are not considered a DMARD drug under this definition.
Creutzfeldt- Jakob Disease	Creutzfeldt-Jacob disease is an incurable brain infection that causes rapidly progressive deterioration of mental function and movement. A neurologist must make a definite diagnosis of Creutzfeldt-Jacob disease based on clinical assessment, EEG and imaging. There must be objective neurological abnormalities on exam along with severe progressive dementia
Fulminant Viral Hepatitis	A submassive to massive necrosis of the liver by any virus, leading precipitously to liver failure. This diagnosis must be supported by all of the following: rapid decreasing of liver size; necrosis involving entire lobules, leaving only a collapsed reticular framework; rapid deterioration of liver function tests; deepening jaundice; and hepatic encephalopathy. Acute Hepatitis infection or carrier status alone, does not meet the diagnostic criteria
Pneumonectomy	The undergoing of surgery on the advice of a consultant medical specialist to remove an entire lung for any physical injury or disease

PART C

1. Benefits under the Rider Policy

1.1 Risk Cover Benefit

Under IndiaFirst Life Group Critical Illness Rider there are 3 benefit options, as mentioned below. Master policyholder / Member can choose any one benefit option on the Rider Policy Commencement Date or on Member Risk Commencement Date, as applicable. The Rider Policy will provide only those benefit cover options which are not available under the Base Policy . Rider Sum Assured cannot exceed the Death Benefit under the Base Policy.

I. Critical Illness (CI) Benefit with 40 conditions
II. Critical Illness (CI) Benefit with 20 conditions
III. Critical Illness (CI) Benefit with 5 conditions

Benefits payable are as below:

I. Critical Illness Benefit with 40 conditions

If the member is diagnosed with any one of the covered critical illnesses out of 40 conditions mentioned below during the Rider Cover Term, the member would receive a lump sum benefit equal to rider Sum Assured . The conditions covered are as below:

Sr. No.	Critical Illness
1	Cancer of specified severity
2	Open Chest CABG
3	Kidney Failure requiring regular dialysis
4	Permanent paralysis of limbs
5	Primary (Idiopathic) Pulmonary Hypertension
6	Myocardial Infarction (First Heart Attack Of Specific Severity)
7	Stroke Resulting in Permanent Symptoms
8	Major organ / bone marrow transplant
9	Multiple Sclerosis with persisting symptoms
10	Surgery to Aorta
11	Apallic Syndrome
12	Benign Brain Tumour
13	Coma of specified severity
14	End Stage Liver Failure
15	End Stage Lung Failure
16	Open Heart Replacement or Repair of Heart Valves
17	Loss of Limbs
18	Blindness
19	Third degree Burns
20	Major Head Trauma
21	Loss of Independent Existence
22	Cardiomyopathy
23	Brain Surgery
24	Alzheimer's Disease
25	Motor Neurone Disease with permanent symptoms
26	Muscular Dystrophy
27	Parkinson's Disease
28	Deafness
29	Loss of Speech
30	Medullary Cystic Disease
31	Systemic Lupus Erythematosus
32	Aplastic Anaemia
33	Poliomyelitis
34	Bacterial Meningitis
35	Encephalitis
36	Progressive Supra nuclear Palsy
37	Severe Rheumatoid Arthritis
38	Creutzfeldt – Jakob Disease
39	Fulminant Viral Hepatitis
40	Pneumonectomy

II. Critical Illness Benefit with 20 conditions

If the member is diagnosed with any one of the covered critical illnesses out of 20 conditions as mentioned below during the Rider Cover Term , the member would receive a lump sum benefit equal to rider Sum Assured . The conditions covered are as below:

Sr. No.	Critical Illness
1	Cancer of specified severity
2	Open Chest CABG
3	Kidney Failure requiring regular dialysis
4	Permanent paralysis of limbs
5	Primary (Idiopathic) Pulmonary Hypertension
6	Myocardial Infarction (First Heart Attack Of Specific Severity)
7	Stroke Resulting in Permanent Symptoms
8	Major organ / bone marrow transplant
9	Multiple Sclerosis with persisting symptoms
10	Surgery to Aorta
11	Apallic Syndrome
12	Benign Brain Tumour
13	Coma of specified severity
14	End Stage Liver Failure
15	End Stage Lung Failure
16	Open Heart Replacement or Repair of Heart Valves
17	Loss of Limbs
18	Blindness
19	Third degree Burns
20	Major Head Trauma

III. Critical Illness Benefit with 5 conditions

If the member is diagnosed with any one of the covered critical illnesses out of 5 conditions as mentioned below during the Rider Cover Term , the member would receive a lump sum benefit equal to rider Sum Assured . The conditions covered are as below:

Sr. No.	Critical Illness
1	Cancer of specified severity
2	Open Chest CABG
3	Kidney Failure requiring regular dialysis
4	Stroke Resulting in Permanent Symptoms
5	Myocardial Infarction (First Heart Attack Of Specific Severity)

Other Terms and Conditions applicable to any of the Three benefit options:

There will be a waiting period of 90 days from the Member Risk Commencement Date, or from date of Revival or reinstatement, if any, whichever is later.

There will be a survival period of 28 days applicable from the date of diagnosis of a critical illness. The Member must survive during this period to become eligible for critical illness benefit payment.

In case of credit linked policies, benefit under this Rider Policy will be applicable for:

i. primary life or main borrower under 100% first claim for joint life cover; and

ii. both co-borrowers under loan sharing basis,

subject to the Rider Sum Assured and Rider Premium being less than the sum assured and premium under the Base Policy.

In case the Rider Cover Term expires and minimum outstanding Base Policy term is more than minimum Rider Cover Term then this Rider Policy will be available at the time of renewal.

In case of a CI claim for any life under co-sharing arrangement, Rider Cover for the Member for whom the CI benefit under this Rider Policy has already been paid, the Rider Cover will cease and will not be renewed further. However, Rider Cover for remaining co-borrower(s), if any, will continue as per terms and conditions of the Rider Policy.

Please note that detailed definition of Critical Illnesses are provided above in Part B.

1.2 Maturity benefit

No maturity benefit will be payable under this Rider Policy.

2. Paid-Up benefits

No Paid-Up benefits will be payable under this Rider Policy.

3. Surrender Benefit

- For One-year renewable Group Term insurance Products : There is no surrender value.
- For Group Credit Life Insurance Products :
  - o Single Premium: Surrender value is acquired immediately. The surrender value for Single Premium in respect of an individual member or master policy holder will be calculated as 50% X Single Rider Premium paid X { 1 – M / P }; Where M = Elapsed months since Member Risk Commencement Date & P = Rider Cover term in month
  - o Limited Premium: Termination / Surrender value is acquired immediately. The termination / surrender value for Limited Premium in respect of an individual member will be calculated as 50% X Rider Premiums Paid X { 1 – M / P }X (Rider Premiums Paid/ Total Rider Premiums payable under the policy
  - o Regular premium: There is no surrender value.

4. Grace Period

Grace period means the specified period of time immediately following the Rider Premium due date during which a payment can be made to renew or continue the Rider Policy in force without loss of continuity benefits such as waiting periods and coverage of pre-existing

diseases. If a valid claim has occurred during grace period, then rider Sum Assured will be paid after deducting due Rider Premium. Grace period terms and conditions will be applicable under Regular/ Limited premium payment options. The grace period will be same as per the base policy.

PART D

5. Premium Payment

Regular Premiums can be paid to us either by monthly/ quarterly/ half yearly/ yearly payment mode, as selected by You/Member for the base Policy. The Premium Payment mode shall be same as the Base Policy to which the Rider Policy is attached. The Rider Premiums should be paid on or before the due dates to avoid any lapsation of this Rider Policy. Grace period terms and conditions will be applicable under Regular/ Limited premium payment options.

Master Policyholder/ Members are advised not to pay insurance premium (initial or renewal) through cash or bearer instrument to any insurance Broker or IndiaFirst Life insurance Advisors /Employees/ Insurance Agents. Insurance Brokers or IndiaFirst Life Insurance Advisors/ Employees / Insurance Agents are not authorized to receive the premium in cash or bearer instrument. Handing over cash or bearer instrument to any insurance Broker or any IndiaFirst Life Insurance Advisor / Employee/ Insurance Agents is solely at Your own risk and the Company in no way be held responsible for any loss in this regard.

Insurance premium cheques must be drawn only in favour of IndiaFirst Life Insurance Company Ltd. (Proposal No. for first premium or Policy No. for renewal premium should be written behind the cheque). Any Cheque payment made shall be deemed to be received by IndiaFirst Life Insurance Company Ltd. only when the same has been received by any office of IndiaFirst Life Insurance Company Ltd. and after an official receipt is issued by the Company.

6. Lapse/ Revival

If the Base Policy lapses, this Rider Policy will cease. If due Rider Premiums are not paid within the Grace Period, the Rider Policy will lapse without acquiring any benefit after the expiry of Grace Period. You may Revive the lapsed Rider Policy within the Revival Period which shall be same as the Base Policy to which the Rider Policy is attached, by:

i. submitting a written request for revival of the lapsed Rider Policy;
ii. paying all unpaid due Rider Premiums without any interest; and
iii. providing a declaration of good health and undergoing a medical examination at your own cost, if needed.

A lapsed Rider Policy will only be revived along with all its benefits in accordance with our underwriting policy. If the Rider Policy is in lapsed status and not revived within revival period or rider cover is opted out from base policy, then inclusion of the rider cover shall be as per Underwriting Policy. In such a case, the Rider Policy will terminate and you will not be entitled to receive any benefits under this Rider Policy. The revival period will be same as per the base policy.

7. Free Look Period

You/Member have a Free Look Period of 30 days starting from the date of receipt of the Rider Policy/COI, whether received electronically or otherwise, to review the terms and conditions of the Rider Policy/COI and in case You/Member disagree with any of those terms or conditions, You/Member shall have the option to return the Policy Document/COI to Us for cancellation, stating the reasons for Your objection, provided no claim has been made under the Rider Policy. In such an event, irrespective of the reason for cancellation You/Member shall be entitled to a refund of the Rider Premium paid subject only to a deduction of a proportionate risk Premium for the period of cover and the expenses incurred by Us on medical examination and stamp duty charges. Such a request received by Us for free look cancellation of the Rider Policy/COI shall be processed and Rider Premium refunded within 7 days of receipt of the request, subject to applicable deductions.

In case of Employer-Employee Groups, only the Master Policyholder, and not the Member, shall have the right to exercise the Free-Look Cancellation option.

8. Loan

No Loan available under this Rider policy.

PART E

9. Charges

This is a non-linked non-participating, group, health, pure risk rider. There are no charges applicable under this Rider Policy.

PART F

10. Making a Claim

You, the Master Policyholder or the member/ nominee/ legal heir/appointee, as the case may be, will give us a written notice of the claim on the occurrence of the covered event. You need to give us all the relevant information in writing to enable us to process the claim as specified in this Rider Policy.

On diagnosis of a covered Critical Illness of the Member, the claimant should intimate the Company in within 90 days from the date of diagnosis of such Critical Illness.

If the claim is not notified to us within the time period specified above, then we shall be provided the reasons for the delay. We shall condone such delay on merits where the delay has been proved to be for genuine reasons beyond the claimant's control.

In case of authorization provided by the Member to the Master Policyholder, at the time of Claim, the Master Policyholder will need to:

- Provide Member Form in respect of the insured Member to whom/whose nominee or beneficiary the claim benefits are payable.
- Provide a confirmation that the Insured Member / Nominee / Beneficiary who had submitted the Claim discharge form is the same person who has been registered by Master Policyholder as the Insured Member / Nominee / Beneficiary under the Group Rider Policy.

Documents required for Critical Illness Rider:

1. Completely filled & signed claim intimation form
2. Medical Certificate confirming the diagnosis of Critical Illness
3. A certificate from an attending Medical Practitioner confirming that the claim does not relate to any Pre-existing Illness or Injury or any Illness or Injury which was diagnosed within the first 90 days of the inception of the Policy.
4. Discharge Card / Indoor consultation paper from the hospital along with the investigation report.
5. First Consultation letter and subsequent prescriptions
6. In the case critical illness arises due to an accident, copy First Information Report, Post Mortem Report & Panchnama, duly attested by police officials or medico-legal certificate.
7. Member's photo, current address proof & photo ID proof
8. Members's copy of first page of bank passbook / statement with bank account details
9. Specific documents listed under the respective critical illness

Any other document or information that we may need to process the claim depending on the cause or nature of the claim. All copies to be self-attested by the Claimant

Claims can be intimated through:

Courier – Intimation with supporting documents can be sent to claims department at head office by courier to the below address.

Claims Department, IndiaFirst Life Insurance Company Ltd, 12th Floor, North {C} wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400063

Email – Intimation with supporting documents can be sent to claims.support@indiafirstlife.com

Website - Visit IndiaFirst website for online claim intimation at https://www.indiafirstlife.com/claims/register-claim-online and follow few simple steps to complete the claim intimation.

WhatsApp – Say Hi on +91 22 6274 9898 and follow few simple steps to complete the claim intimation

11. Exclusions

A. Exclusions for Critical illness benefit:

In addition to the condition specific exclusion mentioned in the definitions, we will not pay any claim arising directly or indirectly due to any of the following causes:

1. Pre-Existing disease - (refer PART B for details);

   After completion of 36 months from Member Risk Commencement Date or reinstatement, as the case may be, pre-existing disease exclusion clause will not be applicable.
2. Intentional self-inflicted injury, attempted suicide while sane or insane.
3. Alcohol or Solvent abuse or taking of Drugs, narcotics or psychotropic substances unless taken in accordance with the lawful directions and prescription of a registered medical practitioner.
4. War, invasion, act of foreign enemy, hostilities (whether war be declared or not), armed or unarmed truce, civil war, mutiny, rebellion, revolution, insurrection, military or usurped power, civil commotion, strikes.
5. Taking part in any naval, military or air force operation during peace time.
6. Participation by the Member in any flying activity, except as a bona fide, fare-paying passenger, pilot, air crew of a recognized airline on regular routes and on a scheduled timetable
7. Participation by the Member in a criminal or unlawful act with a criminal intent.
8. Engaging in or taking part in professional sport(s) or any hazardous pursuits, including but not limited to, diving or riding or any kind of race; underwater activities involving the use of breathing apparatus or not; martial arts; hunting; mountaineering; parachuting; bungee-jumping.
9. Any external congenital anomaly.

Congenital Anomaly which is in the visible and accessible parts of the body and present since birth, and which is abnormal with reference to form, structure or position is called External Congenital Anomaly.

10. Nuclear Contamination; the radioactive, explosive or hazardous nature of nuclear fuel materials or property contaminated by nuclear fuel materials or accident arising from such nature

12. Nomination shall be governed as per section 39 of the Insurance Act, 1938 as amended from time to time.

Nomination of a life insurance Policy may be made is as below in accordance with Section 39 of the Insurance Act, 1938 as amended from time to time or any other Regulation/Circular as issued by the Authority. A simplified version of the provisions of Section 39 is provided below:

1) The Policyholder of a life insurance on his own life shall nominate a person or persons to whom money secured by the Policy shall be paid in the event of his death. Nomination is mandatory to facilitate payment of claim amount in case of death of life assured
2) Where the nominee is a minor, the Policyholder shall appoint any person in the manner laid down by the insurer to receive the money secured by the Policy in the event of Policyholder's death during the minority of the nominee.
3) Nomination may be incorporated in the text of the Policy itself or may be endorsed on the Policy communicated to the insurer and can be registered by the insurer in the records relating to the Policy.
4) Nomination can be changed at any time before Policy matures, by an endorsement or a further endorsement or a will as the case may be.
5) A notice in writing of Change of nomination must be delivered to the insurer Otherwise, insurer will not be liable for any payment under the Policy made bonafide by the insured to the nominee unless it is mentioned in the text of the Policy or in the registered records of the insurer.
6) Fee for registering cancellation or change in the nomination may be required to be paid to the insurer which shall not exceed INR 100 (INR One hundred only) on each occasion or as may be specified by the Authority through Regulations.
7) On receipt of notice with fee, the insurer should grant a written acknowledgement to the Policyholder of having registered a nomination or change thereof.
8) A transfer or assignment made in accordance with Section 38 shall automatically cancel the nomination except in case of assignment to the insurer or other transferee or assignee for purpose of loan or against security or its reassignment after repayment. In such case, the nomination will not get cancelled to the extent of insurer's or transferee's or assignee's interest in the Policy. The nomination will get revived on repayment of the loan.
9) The right of any creditor to be paid out of the proceeds of any Policy of life insurance shall not be affected by the nomination.
10) In case of nomination by Policyholder whose life is insured, if the nominees die before the Policyholder, the proceeds are payable to Policyholder or his heirs or legal representatives or holder of succession certificate.
11) In case nominee(s) survive the person whose life is insured, the amount secured by the Policy shall be paid to such survivor(s).
12) Where the Policyholder whose life is insured nominates his (a) parents or (b) spouse or (c) children or (d) spouse and children (e) or any of them; the nominees are beneficially entitled to the amount payable by the insurer to the Policyholder unless it is proved that Policyholder could not have conferred such beneficial title on the nominee having regard to the nature of his title.
13) If nominee(s) die after the Policyholder but before his share of the amount secured under the Policy is paid, the share of the expired nominee(s) shall be payable to the heirs or legal representative of the nominee or holder of succession certificate of such nominee(s).
14) The provisions of sub-section 7 and 8 (13 and 14 above) shall apply to all life insurance policies maturing for payment after the commencement of Insurance Laws (Amendment) Act, 2015 (i.e. 23.03.2015).
15) If Policyholder dies after maturity but the proceeds and benefit of the Policy has not been paid to him because of his death, his nominee(s) shall be entitled to the proceeds and benefit of the Policy.
16) The provisions of Section 39 are not applicable to any life insurance Policy to which Section 6 of Married Women's Property Act, 1874 applies or has at any time applied except where before or Insurance Laws (Amendment) Act, 2015, a nomination is made in favour of spouse or children or spouse and children whether or not on the face of the Policy it is mentioned that it is made under Section 39. Where nomination is intended to be made to spouse or children or spouse and children under Section 6 of MWP Act, it should be specifically mentioned on the Policy. In such a case only, the provisions of Section 39 will not apply.

[Disclaimer: This is not the exact text of Section 39 of the Insurance Act, 1938 and only a simplified version prepared for general information. Policyholders are advised to refer to the Insurance Act, 1938 and any other applicable Regulation/Circulars issued by the Authority for complete and accurate details.]

13. Assignment shall be governed as per section 38 of the Insurance Act, 1938 as amended from time to time.

Assignment or transfer of a Policy should be in accordance with Section 38 of the Insurance Act, 1938 as amended time to time and any other applicable Regulation/Circulars issued by the Authority. A simplified version of the provisions of Section 38 is provided below:

1) This Policy may be transferred/assigned, wholly or in part, with or without consideration.

2) An Assignment may be effected in a Policy by an endorsement upon the Policy itself or by a separate instrument under notice to the Insurer.

3) The instrument of assignment should indicate the fact of transfer or assignment and the reasons for the assignment or transfer, antecedents of the assignee and terms on which assignment is made.

4) The assignment must be signed by the transferor or assignor or duly authorized agent and attested by at least one witness.

5) The transfer or assignment shall not be operative as against an insurer until a notice in writing of the transfer or assignment and either the said endorsement or instrument itself or copy there of certified to be correct by both transferor and transferee or their duly authorized agents have been delivered to the insurer.

6) Fee for granting a written acknowledgement of the receipt of notice of assignment or transfer assignment may require to be paid to the insurer which shall not exceed INR 100 (INR One hundred only) or as may be specified by the Authority

7) On receipt of notice with fee, the insurer should Grant a written acknowledgement of receipt of notice. Such notice shall be conclusive evidence against the insurer of duly receiving the notice.

8) If the insurer maintains one or more places of business, such notices shall be delivered only at the place where the Policy is being serviced.

9) The insurer may accept or decline to act upon any transfer or assignment or endorsement, if it has sufficient reasons to believe that it is (a) not bonafide or (b) not in the interest of the Policyholder or (c) not in public interest or (d) is for the purpose of trading of the insurance Policy.

10) Before refusing to act upon endorsement, the Insurer should record the reasons in writing and communicate the same in writing to Policyholder within 30 days from the date of Policyholder giving a notice of transfer or assignment.

11) In case of refusal to act upon the endorsement by the Insurer, any person aggrieved by the refusal may prefer a claim to IRDAI within 30 days of receipt of the refusal letter from the Insurer.

12) The priority of claims of persons interested in an insurance Policy would depend on the date on which the notices of assignment or transfer is delivered to the insurer; where there are more than one instruments of transfer or assignment, the priority will depend on dates of delivery of such notices. Any dispute in this regard as to priority should be referred to Authority.

13) Every assignment or transfer shall be deemed to be absolute assignment or transfer and the assignee or transferee shall be deemed to be absolute assignee or transferee, except

   a. where assignment or transfer is subject to terms and conditions of transfer or assignment OR

   b. where the transfer or assignment is made upon condition that

      i. the proceeds under the Policy shall become payable to Policyholder or nominee(s) in the event of assignee or transferee dying before the insured OR

      ii. the insured surviving the term of the Policy.

   Such conditional assignee will not be entitled to obtain a loan on Policy or surrender the Policy. This provision will prevail notwithstanding any law or custom having force of law which is contrary to the above position.

14) In other cases, the insurer shall, subject to terms and conditions of assignment, recognize the transferee or assignee named in the notice as the absolute transferee or assignee and such person

   a. shall be subject to all liabilities and equities to which the transferor or assignor was subject to at the date of transfer or assignment and

   b. may institute any proceedings in relation to the Policy

   c. obtain loan under the Policy or surrender the Policy without obtaining the consent of the transferor or assignor or making him a party to the proceedings

15) Any rights and remedies of an assignee or transferee of a life insurance Policy under an assignment or transfer effected before commencement of the Insurance Laws (Amendment) Act, 2015 shall not be affected by this section.

[Disclaimer: This is not the exact text of Section 38 of the Insurance Act, 1938 and only a simplified version prepared for general information. Policyholders are advised to refer to the Insurance Act, 1938 and any other applicable Regulation/Circulars issued by the Authority for complete and accurate details.]

14. Rider Benefit Ceases/ Ends/ Terminates

The rider benefit will terminate on the happening of the earliest of any of the following:

- On the date of acceptance of free-look cancelation request from the Member / Master Policyholder to the company
- On payment of rider Sum Assured against a valid claim
- Once a claim is repudiated for a particular Critical Illness (CI) due to material non-disclosure, then the rider policy will be null and void. However, if a CI claim is rejected due to any other terms & conditions, then the CI Rider Cover will continue for any of the remaining Critical Illnesses
- On non-payment of due Limited/ Regular premium for base policy/ Rider Policy, within the grace period as applicable
- On the expiry of the Revival Period
- On the expiry of Rider Cover term for Member
- On the maturity of the base policy or the date on which the base policy/COI issued under the Base Policy is surrendered or is terminated by Master Policyholder/Member
- On the expiry of Policy Term of the base policy. At the time of Rider Policy attachment to the base policy,

rider Cover Term/ Rider Premium Paying Term would be aligned to the outstanding term/Premium Paying Term of the base policy.

- On receipt of written request for cancelation / surrender of this Rider Policy/COI, effective from the next Rider Premium due date provided no surrender / termination value has been paid.
- On cancelation/termination of this Rider Policy on grounds of misrepresentation, fraud or non-disclosure by member/Master Policyholder as per section 45 of Insurance Act (1938) as amended from time to time

15. Change of Address

You are required to inform us in writing, about any change in your/ Member's/ Nominee(s)'s address with address proof. This will ensure that our correspondence reaches you/Member/ the Nominee(s) without any delay. We will not be liable on account of your failure to up-date your current address in our records or registering an address with us which is incorrect.

16. Disclosures

Misrepresentation/Fraudulent Disclosures: shall be governed as per Section 45 of Insurance Act, 1938 as amended from time to time.

The Age of the Member is derived basis the declaration in the Proposal Form. Due to misstatement of Age if You have paid less Premium(s) than what was payable for the correct Age, We will charge and You the difference in Premium since the Policy Commencement Date without interest or deduct the same from the Policy benefit payable. If the date of birth of the Member has been misstated resulting in higher Premium(s) paid by You than what was payable for the correct Age, We will refund the excess Premiums without any interest. If at the correct Age, the Member was not insurable according to Our requirements, We reserve the right to refund the Premiums paid till date post deduction of any relevant cost, expenses or charges as applicable and terminate the Policy in accordance with Section 45 of the Insurance Act, 1938, as amended from time to time.

Section 45 of the Insurance Act, 1938 as amended from time to time:

Fraud and Mis-statement shall be dealt as per the provisions of Section 45 of Insurance Act, 1938. A simplified version of the provisions of Section 45 is provided below:

1. No Policy of Life Insurance shall be called in question on any ground whatsoever after expiry of 3 years from
   a. the date of issuance of Policy or
   b. the date of commencement of risk or
   c. the date of revival of Policy or
   d. the date of rider to the Policy whichever is later.
2. On the ground of fraud, a Policy of Life Insurance may be called in question within 3 years from
   a. the date of issuance of Policy or
   b. the date of commencement of risk or
   c. the date of revival of Policy or
   d. the date of rider to the Policy whichever is later.

For this, the insurer should communicate in writing to the insured or legal representative or nominee or assignees of insured, as applicable, mentioning the ground and materials on which such decision is based.

3. Fraud means any of the following acts committed by insured or by his agent, with the intent to deceive the insurer or to induce the insurer to issue a life insurance Policy:
   a. The suggestion, as a fact of that which is not true and which the insured does not believe to be true;
   b. The active concealment of a fact by the insured having knowledge or belief of the fact;
   c. Any other act fitted to deceive; and
   d. Any such act or omission as the law specifically declares to be fraudulent.
4. Mere silence is not fraud unless, depending on circumstances of the case, it is the duty of the insured or his agent keeping silence to speak or silence is in itself equivalent to speak.
5. No Insurer shall repudiate a life insurance Policy on the ground of Fraud, if the Insured / beneficiary can prove that the misstatement was true to the best of his knowledge and there was no deliberate intention to suppress the fact or that such mis-statement of or suppression of material fact are within the knowledge of the insurer. Onus of disproving is upon the Policyholder, if alive, or beneficiaries.
6. Life insurance Policy can be called in question within 3 years on the ground that any statement of or suppression of a fact material to expectancy of life of the insured was incorrectly made in the proposal or other document basis which Policy was issued or revived or rider issued. For this, the insurer should communicate in writing to the insured or legal representative or nominee or assignees of insured, as applicable, mentioning the ground and materials on which decision to repudiate the Policy of life insurance is based.
7. In case repudiation is on ground of mis-statement and not on fraud, the premium collected on Policy till the date of repudiation shall be paid to the insured or legal representative or nominee or assignees of insured, within a period of 90 days from the date of repudiation.
8. Fact shall not be considered material unless it has a direct bearing on the risk undertaken by the insurer. The onus is on insurer to show that if the insurer had been aware of the said fact, no life insurance Policy would have been issued to the insured.
9. The insurer can call for proof of age at any time if he is entitled to do so and no Policy shall be deemed to be called in question merely because the terms of the Policy are adjusted on subsequent proof of age of life insured. So, this Section will not be applicable for questioning age or adjustment based on proof of age submitted subsequently.

[Disclaimer: This is not the exact text of Section 45 the Insurance Act, 1938 and only a simplified version prepared for general information. Policyholders are advised to refer to the Insurance Act, 1938 for complete and accurate details.]

17. Right to Revise/ Delete/ Alter the Terms and Conditions of this Policy

We may revise, delete and/ or alter any of the terms and conditions of this Rider Policy, by sending a prior written notice of 30 (Thirty) days, subject to receipt of prior approval of the Regulatory Authority or subsequent changes/amendments in the applicable laws.

18. Governing Law and Jurisdiction

All claims, disputes or differences under this Rider Policy will be governed by Indian laws and shall be subject to the jurisdiction of Indian Courts.

20. Turn Around Time for various servicing request and claims processing are as mentioned below:

- We shall settle or reject a claim, as may be the case, within 15 days of the receipt of the last necessary document
  - o In the case of delay in the payment of a claim, we shall pay interest from the date of receipt of last necessary document to the date of payment of claim at a rate 2% above the bank rate
- However, where the circumstances of a claim warrant an investigation in our opinion, we shall initiate and complete such investigation at the earliest, in any case not later than 30 days from the date of receipt of last necessary document. In such cases, we shall settle the claim within 45 days from the date of receipt of last necessary document.
  - o In case of delay beyond stipulated 45 days we shall pay interest at a rate 2% above the bank rate from the date of receipt of last necessary document to the date of payment of claim.

Policy Servicing TAT's	
Full Surrender	7 Days
Freelook Cancellation	7 Days
Request for Refund of Proposal Deposit	15 days
Refund of outstanding proposal deposit	30 days

20. Issue of duplicate Policy Document

If the Policy Document is lost or destroyed, then, the Master Policyholder can submit to us the filled in 'Indemnity Bond for Loss of Policy Document' form, which is available on our website.

We will issue a duplicate Policy Document duly endorsed to show that it is being issued following the loss or destruction of the original Policy Document.

The Company will not charge any additional fee for the issuance of duplicate Policy Document. Currently, only Stamp Duty fee (as applicable for the applicable State/Union-Territory) is being charged.

Upon the issue duplicate Policy Document, the original Policy Document will cease to have any legal effect.

PART G

21. Policy Servicing & Grievance Handling Mechanism

You may contact us in case of any grievance at any of our branches or at Customer Care, IndiaFirst Life Insurance Company Ltd, 12th & 13th floor, North [C] Wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400 063, Contact No.: 1800 209 8700, Email id: customer.first@indiafirstlife.com. IRDAI Regn. No. 143. CIN: U66010MH2008PLC183679

a. An acknowledgment to all such grievances received will be sent immediately from the date of receipt of the grievance

b. A written communication giving reasons of either redressing or rejecting the grievance will be sent to you within 14 days from the date of receipt of the grievance. In case We don't receive a revert from You within 8 weeks from the date of registration of grievance, We will treat the complaint as closed.

c. However, if you are not satisfied with our resolution provided or have not received any response within 14 , then, you may approach our Grievance Officer at the nearest IndiaFirst Life Insurance's branch or you may write to our Grievance Redressal Officer at grievance.redressal@indiafirstlife.com.

d. If you are not satisfied with the resolution or have not received any response within 14 days then you can contact the insurance ombudsman. For the list of ombudsman office please refer Annexure B.

e. Further, you may approach the Grievance Cell of the Insurance Regulatory and Development Authority of India (IRDAI) on the following contact details:

IRDAI Grievance Call Centre (IGCC) TOLL FREE NO: 155255/ 1800 4254 732

Email ID: complaints@irdai.gov.in

You can also register your complaint online athttps://bimabharosa.irdai.gov.in/

Address for communication for complaints by post:

Policyholder Protection & Grievance Redressal Department (PPGR) – Grievance Redressal Cell,

Insurance Regulatory and Development Authority of India,

Sy. No. 115/1, Financial District, Nanakramguda

Gachibowli, Hyderabad- 500032, Telangana

IRDAI TOLL FREE NO: 18004254732

Insurance Ombudsman

In case you are dissatisfied with the decision/resolution of the Company, you may approach the Insurance Ombudsman located nearest to you (please refer to or visit our website www.indiafirstlife.com) if your grievance pertains to :

- Delay in settlement of claims, beyond the time specified in the regulations, framed under the Insurance Regulatory and Development Authority Act, 1999;
- any partial or total repudiation of claims by the life insurer, general insurer or health insurer;
- disputes over premium paid or payable in terms of insurance policy;
- misrepresentation of policy terms and conditions at any time in the policy document or policy contract;
- legal construction of insurance policies in so far as the dispute relates to claim;
- policy servicing related grievances against insurers and their agents and intermediaries;
- issuance of life insurance policy, general insurance policy including health insurance policy which is not in conformity with the proposal form submitted by the proposer;
- non issuance of insurance policy after receipt of premium in life insurance and general insurance including health insurance; and

any other matter resulting from the violation of provisions of the Insurance Act, 1938 as amended from time to time or the regulations, circulars, guidelines or instructions issued by IRDAI from time to time or the terms and conditions of the policy contract, in so far as they relate to issues mentioned in clauses above.

The complaint should be made in writing and the same should be duly signed by the complainant or by his legal heirs, nominee or assignee with full details of the complaint and the contact information of the complainant.

As per provision 14 of the Insurance Ombudsman Rules, 2017, the complaint to the Ombudsman can be made by you or the complainant, within a period of 1 (One) year from the date of rejection of the grievance by Us or after receipt of decision which is not to your satisfaction or after expiry of one month from the date of sending representation to Us if We fail to furnish reply to You provided the same dispute is not already decided by or pending before or disposed of by any court or consumer forum or arbitrator.

f. All grievances arising out of or in relation to claims/issues arising out this agreement shall only be raised via the grievance redressal mechanism available on the website of the Company and / or agreed in this Agreement. It is clarified that no grievances shall be entertained in case the same are addressed to the personal email ids of the CEO/board of directors of the company. Rather any such unwarranted emails would be classified as spam and the Company may take appropriate legal actions against such acts of spamming.

Disclaimers

Applicable taxes levied as per extant tax laws shall be deducted from the premium or from the allotted units as applicable.

IndiaFirst Life Insurance Company Limited, IRDAI Regn No.143, CIN: U66010MH2008PLC183679, Address: 12th & 13th floor, North [C] Wing, Tower 4, Nesco IT Park, Nesco Center, Western Express Highway, Goregaon (East), Mumbai – 400 063. www.indiafirstlife.com, SMS <LIFE> to 5667735 SMS Charges apply. Toll free No – 1800 209 8700. Trade logo of promoters displayed above belongs to M/s Bank of Baroda and is used by IndiaFirst Life Insurance Co. Ltd. under license.

Annexure B - List of Ombudsmen

Office of the Insurance Ombudsman - Ahmedabad Jeevan Prakash Building , 06th Floor, Tilak Marg, Relief Road, AHMEDABAD- 380001 Tel. 079- 25501201/02/05/06 Email: bimalokpal.ahmedabad@ecoi.co.in Area of Jurisdiction - Gujarat, Dadra & Nagar Haveli, Daman and Diu	Office of the Insurance Ombudsman - Bhopal Janak Vihar Complex, 2nd Floor, 6, Malviya Nagar, Opp. Airtel Office, Near New Market, BHOPAL – 462 003.Tel.: 0755 - 2769201 / 2769202 Fax: 0755 - 2769203 Email: bimalokpal.bhopal@ecoi.co.in Area of Jurisdiction - Madhya Pradesh & Chhattisgarh
Office of the Insurance Ombudsman – Bhubaneshwar 62, Forest Park, BHUBNESHWAR – 751 009. Tel.: 0674 - 2596461 /2596455 Fax: 0674 – 2596429 Email: bimalokpal.bhubaneswar@ecoi.co.in Area of Jurisdiction - Odisha	Office of the Insurance Ombudsman – Chandigarh S.C.O. No. 101, 102 & 103, 2nd Floor, Batra Building, Sector 17 – D, CHANDIGARH – 160 017. Tel.: 0172 - 2706196 / 2706468 Fax: 0172 – 2708274 Email: bimalokpal.chandigarh@ecoi.co.in Area of Jurisdiction - Punjab, Haryana, Himachal Pradesh, Jammu & Kashmir, Chandigarh
Office of the Insurance Ombudsman – Chennai Fatima Akhtar Court, 4th Floor, 453, Anna Salai, Teynampet, CHENNAI – 600 018. Tel.: 044 - 24333668 / 24335284 Fax: 044 - 24333664 Email: bimalokpal.chennai@ecoi.co.in Area of Jurisdiction - Tamil Nadu, –Pondicherry Town and Karaikal (which are part of Pondicherry)	Office of the Insurance Ombudsman – New Delhi 2/2 A, Universal Insurance Building, Asaf Ali Road, NEW DELHI – 110 002. Tel.: 011 - 23239633 / 23237532 Fax: 011 – 23230858 Email: bimalokpal.delhi@ecoi.co.in Area of Jurisdiction - Delhi
Office of the Insurance Ombudsman – Guwahati, Jeevan Nivesh, 5th Floor, Nr. Panbazar over bridge, S.S. Road, GUWAHATI – 781001 (ASSAM). Tel.: 0361 - 2132204 / 2132205 Fax: 0361 – 2732937 Email: bimalokpal.guwahati@ecoi.co.in Area of Jurisdiction - Assam, Meghalaya, Manipur, Mizoram, Arunachal Pradesh,Nagaland and Tripura	Office of the Insurance Ombudsman – Hyderabad, 6-2-46, 1st floor, "Moin Court", Lane Opp. Saleem Function Palace, A. C. Guards, Lakdi-Ka-Pool, HYDERABAD - 500 004. Tel.: 040 - 65504123 / 23312122 Fax: 040 – 23376599 Email: bimalokpal.hyderabad@ecoi.co.in Area of Jurisdiction - Andhra Pradesh, Telangana, Yanam and part of Territory of Pondicherry
Office of the Insurance Ombudsman - Ernakulam 2nd Floor, Pulinat Bldg., Opp. Cochin Shipyard, M. G. Road, ERNAKULAM - 682 015. Tel.: 0484 - 2358759 / 2359338 Fax: 0484 – 2359336 Email: bimalokpal.ernakulam@ecoi.co.in Area of Jurisdiciton - Kerala, Lakshadweep, Mahe – a part of Pondicherry	Office of the Insurance Ombudsman - Kolkata Hindustan Bldg. Annexe, 4th Floor, 4, C.R. Avenue, KOLKATA - 700 072. Tel.: 033 - 22124339 / 22124340 Fax : 033 – 22124341 Email: bimalokpal.kolkata@ecoi.co.in Area of Jurisdiction - West Bengal, Sikkim, Andaman & Nicobar Islands
Office of the Insurance Ombudsman – Lucknow, 6th Floor, Jeevan Bhawan, Phase-II, Nawal Kishore Road, Hazratganj, LUCKNOW - 226 001. Tel.: 0522 - 2231330 / 2231331 Fax: 0522 – 2231310 Email: bimalokpal.lucknow@ecoi.co.in Area of Jurisdiction - Districts of Uttar Pradesh : Laitpur, Jhansi, Mahoba, Hamirpur, Banda, Chitrakoot, Allahabad, Mirzapur, Sonbhabdra, Fatehpur, Pratapgarh, Jaunpur,Varanasi,Gazipur, Jalaun, Kanpur, Lucknow, Unnao, Sitapur, Lakhimpur, Bahraich, Barabanki, Raebareli,Sravasti, Gonda, aizabad, Amethi,Kaushambi, Balrampur, Basti, Ambedkarnagar, ultanpur, Maharajgang, Santkabirnagar, Azamgarh, Kushinagar, Gorkhpur, Deoria, Mau, Ghazipur, Chandauli, Ballia, Sidharathnagar	Office of the Insurance Ombudsman – Noida Bhagwan Sahai Palace, 4th Floor, Main Road, Naya Bans, Sector 15, Distt: Gautam Buddh Nagar, UTTAR PRADESH (U.P.) - 201301. Tel.: 0120-2514250 / 2514252 / 2514253 Email: bimalokpal.noida@ecoi.co.in Area of Jurisdiction - State of Uttaranchal and the following Districts of Uttar Pradesh: Agra, Aligarh, Bagpat, Bareilly, Bijnor, Budaun, Bulandshehar, Etah, Kanooj, Mainpuri, Mathura, Meerut, Moradabad, Muzaffarnagar, Oraiyya, Pilibhit, Etawah, Farrukhabad, Firozbad, Gautam Buddh Nagar, Ghaziabad, Hardoi, Shahjahanpur, Hapur, Shamli, Rampur, Kashganj, Sambhal, Amroha, Hathras, Kanshiramnagar, Saharanpur

Office of the Insurance Ombudsman – Jaipur, Jeevan Nidhi – II Bldg., Gr. Floor, Bhawani Singh Marg, JAIPUR - 302 005. Tel.: 0141 - 2740363 Email: bBimalokpal.jaipur@gbic.co.inecoi.co.in Area of Jurisdiction - Rajasthan	Office of the Insurance Ombudsman – Pune, Jeevan Darshan Bldg., 3rd Floor, C.T.S. Nos. 195 to 198, N.C. Kelkar Road, Narayan Peth, PUNE – 411 030. Tel.: 020-41312555 Email: bimalokpal.pune@ecoi.co.in Area of Jurisdiction - Maharashtra, Area of Navi Mumbai and Thane excluding Mumbai Metropolitan Region
Office of the Insurance Ombudsman - Bengaluru Jeevan Soudha Building, PID No. 57-27-N-19 Ground Floor, 19/19, 24th Main Road, JP Nagar, Ist Phase, BENGALURU – 560 078. Tel.: 080 - 26652048 / 26652049 Email: bimalokpal.bengaluru@ecoi.co.in Area of Jurisdiction - Karnataka	Office of the Insurance Ombudsman – Mumbai 3rd Floor, Jeevan Seva Annexe, S. V. Road, Santacruz (W), MUMBAI - 400 054. Tel.: 022 - 26106552 / 26106960 Fax: 022 - 26106052 Email: bimalokpal.mumbai@ecoi.co.in Area of Jurisdiction – Goa, Mumbai Metropolitan Region excluding Navi Mumbai & Thane
Office of the Insurance Ombudsman – Patna 1st Floor, Kalpana Arcade Building, Bazar Samiti Road, Bahadurpur, PATNA – 800006 Tel No: 0612-2680952 Email id : bimalokpal.patna@gbic.co.in. Area of Jurisdiction - Bihar, Jharkhand	



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